

Chemotherapy for bowel cancer

Chemotherapy uses anti-cancer drugs to treat bowel cancer. Bowel cancer is a general term for cancer that starts in the colon or rectum.

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Chemotherapy for colon or rectal cancer

[Chemotherapy](#) uses anti-cancer (cytotoxic) drugs to destroy cancer cells. It is sometimes called systemic anti-cancer therapy (SACT). Chemotherapy drugs disrupt the way cancer cells grow and divide. But they also affect normal cells.

Your doctors may offer you chemotherapy in the following situations:

- After surgery, to reduce the risk of cancer coming back. This is called adjuvant therapy.
- Before surgery:
 - to shrink the cancer and reduce the risk of it coming back
 - if you are having cancer removed from the liver or lungs.
- As the main treatment, if the cancer has spread to other parts of the body, such as the liver or lungs. This aims to control it for as long as possible.
- With radiotherapy to treat rectal cancer. This is called [chemoradiation](#).

Booklets and resources



Booklets

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Understanding chemotherapy

Chemotherapy drugs for colon and rectal cancer

The drugs most commonly used to treat [colon cancer](#) or [rectal cancer](#) are:

- [fluorouracil \(5FU\)](#), usually given with folinic acid (leucovorin)
- [capecitabine \(Xeloda[®]\)](#).
- [oxaliplatin \(Eloxatin[®]\)](#).
- [irinotecan \(Campto[®]\)](#).
- [trifluridine and tipiracil \(Lonsurf[®]\)](#).
- [raltitrexed \(Tomudex[®]\)](#).

Often, 2 or more chemotherapy drugs are given in combination. The most common combinations are:

- [FOLFOX](#) (folinic acid, fluorouracil and oxaliplatin)
- [CAPOX \(XELOX\)](#) (capecitabine and oxaliplatin)
- [FOLFIRI](#) (folinic acid, fluorouracil and irinotecan)
- [FOLFOXIRI](#) (folinic acid, fluorouracil, oxaliplatin and irinotecan).

How chemotherapy is given

You usually have chemotherapy as an outpatient. Some people need to be in hospital for a few days to have their treatment.

You may have chemotherapy into a vein (intravenously) or as a tablet. If you have chemotherapy into a vein, it goes directly into your blood. The blood carries it to all areas of your body. If you have chemotherapy as a tablet, it is absorbed into the blood and also reaches all areas of your body.

Before you start treatment, you may have a blood test to check whether you have low levels of an enzyme called DPD. People who have low DPD levels can have more serious side effects with 2 of the chemotherapy drugs used to treat bowel cancer. These are fluorouracil (5FU) and capecitabine.

We have more information about [how chemotherapy is given](#).

Getting support

Macmillan is here to support you. If you would like to talk, you can:

- Call the Macmillan Support Line for free on [0808 808 00 00](#).
- [Chat to our specialists online](#).
- Visit our [chemotherapy forum](#) to talk with people who have had chemotherapy, share your experience, and ask questions.

Side effects of chemotherapy for bowel cancer

Chemotherapy drugs can cause different side effects. Many of these can be managed with different medicines and most of them will get better after treatment ends. Chemotherapy affects everyone differently. Some people may have only a few side effects whereas others may have more.

Your cancer doctor or nurse will tell you more about what to expect. Tell them about any side effects you have, as they can usually be well controlled with medicines.

Although they can be hard to cope with, side effects usually get better after your treatment finishes.

We describe some of the common side effects here. We have more information about [chemotherapy side effects](#).

Risk of infection

Chemotherapy can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number you have been given:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) a lot, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

Chemotherapy can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots on the skin that may look like a rash.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (reduced number of red blood cells)

Chemotherapy can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and lightheaded.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Diarrhoea

Some chemotherapy drugs can cause [diarrhoea](#). Diarrhoea means passing more stools than is usual for you, or having watery or loose stools. If you have a [stoma](#), it will be more active than usual.

Sometimes diarrhoea can be severe. Your hospital team may give you anti-diarrhoea drugs to take at home. It is important to follow any advice from them about taking anti-diarrhoea drugs. Contact the hospital on the 24-hour number straight away if:

- you have diarrhoea at night
- you have uncomfortable stomach cramps
- you have diarrhoea 4 or more times in a day
- you have a moderate increase in stoma activity
- the anti-diarrhoea drugs do not work within 24 hours.

If you are taking [capecitabine](#) tablets, the hospital may ask you to stop taking them. They will tell you when you can start taking them again.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control [sickness](#). Take the drugs exactly as they tell you, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

Sore mouth

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals. Contact the hospital straight away on the 24-hour number if:

- your mouth or throat is sore, or affecting how much fluid you can drink or food you can eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

Tiredness (fatigue)

[Feeling tired](#) is a common side effect. It is often worse towards the end of treatment and for some weeks after it ends. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can give you more energy. If you feel sleepy, do not drive or use machinery.

Throat spasm

Rarely, [oxaliplatin](#) can cause a spasm in the throat area around the voicebox (larynx). This can cause difficulties with swallowing and breathing. This might happen during treatment, or in the first few days after treatment.

This side effect can be frightening, but it should only be temporary. If you have breathing difficulties, take long, deep breaths through your nose. This will have a calming effect and help your breathing return to normal.

This symptom may be worse in cold temperatures. It may help to avoid cold drinks during treatment, and for a few days afterwards. It may also help to wrap up warm and cover your nose and mouth in cold weather.

It is important to tell your doctor if you have this side effect. They may give the next doses of oxaliplatin over 4 to 6 hours to reduce the chance of throat spasms.

Numb or tingling hands or feet

Some chemotherapy drugs can affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This side effect is called [peripheral neuropathy](#). You might find it hard to do fiddly tasks such as fastening buttons or tying shoe laces. If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug.

Hair loss

Many of the chemotherapy drugs for colon and rectal cancer do not cause hair loss, but you may still get some hair thinning. The chemotherapy drug [irinotecan](#) can cause hair loss. Your hair may get thinner or you may lose all the hair from your head.

Your nurse can talk to you about ways to [cope with hair loss](#). Hair loss is almost always temporary. Your hair will usually grow back after treatment ends.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

- be sore
- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

Effects on the heart

[Fluorouracil](#) and [capecitabine](#) can [affect the way the heart works](#). You may have tests to see how well your heart is working. These may be done before, during and after treatment.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat
- swollen feet and ankles.

Always call 999 if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- dizziness
- difficulty breathing.

Effects on sex life

Having chemotherapy can sometimes affect your sex life. Side effects like tiredness and feeling sick can reduce your sex drive and make having sex difficult.

If you have a partner, try to tell them how you feel. It can also help to talk with your doctor or specialist nurse about your concerns. They can reassure you and support you with any problems. We have more information about [cancer and your sex life](#).

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Fertility

Some cancer drugs can affect whether you can get pregnant or make someone pregnant. If you are worried about this, it is important to talk with your doctor before you start treatment.

We have more information about [fertility and cancer](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our bowel cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

National Institute for Health and Care Excellent (NICE). Colorectal cancer. NICE guideline [NG151]. Updated December 2021. Available from: www.nice.org.uk/guidance/NG151 [accessed Jan 2023].

Cervantes A, Adam R, Rosello S, et al. Metastatic colorectal cancer: ESMO Clinical Practice Guideline for diagnosis, treatment and follow-up. Annals of Oncology. 2023;34(1):10-32. Available from: www.annalsofoncology.org [accessed Jan 2023].

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Bowel (colon and rectal) cancer forum

An anonymous network of people affected by colon, rectal, small bowel, or other colorectal cancers, which is free to join. Share experiences, ask questions and talk to people who understand.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help